POSITION DESCRIPTION

Agency PD Number PX3439 PCMC

2a. Reason for Submission	3. Service	4. Employing Office Location	5. Duty Station
☐ Redescription ☐ New ☐ Update Supervisor Certification ☐ Reestablishment ☐ Other ☑ Reorganization	☐ Headquarters ☑ Field	Phoenix, Arizona	PAIHS - PIMC
2b. Position Information (Complete all that apply) ☐ PCA ☐ Drug Testing ☐ Physical Exam Required	6. Fair Labor Standards Act ☐ Exempt ☑ Nonexempt	7. Financial Statements Required ☐ OGE-278 ☐ OGE-450 ☑ N/A	8. Subject to IA ☑ Yes ☐ No
Acq Code: 00 BUS Code: 1776 Cyber Sec Code: 00 FPL: 10 ☐ Select Agent Access ☐ Incumbency Only ☐ Commissioned Corps	9. Position Status ☑ Competitive ☐ Excepted (*Specify in remarks*) ☐ SES (GEN) ☐ SES (CR)	11. Position is: ☐ Supervisor/Manager (Code 2) ☐ Supervisor-CSRA (Code 4) ☐ Management Official (CSRA) (Code 5) ☐ Work Leader (Code 6) ☐ Team Leader (Code 7) ☑ Neither (Code 8)	12. Position Sensitivity ☐ 1-Nonsensitive ☐ 2-Noncritical Sensitive ☐ 3-Critical Sensitive ☐ 4-Special Sensitive ☑ 5-Public Trust/ Moderate ☐ 6-Public Trust/High Risk
2c. Explanation (Show any positions replaced) Realigned under Nursing Services. Replaces PX3439 PCMC, Nurse, GS-0610-10, dated 12/10/2019.	10a. Administrative Code GFLM262		13. Comp Level NUG0
			14. Job Code PX3439
☐ Limited Term ☐ Limited Emergency NTE Date:	10b. Job Requisition/Position Number		15. OPM Certification No.

16. Classified/Graded by:	Official Title of Position	Pay Plan	Occupational Series Code	Grade	Initials	Date
a. U.S. Office of Personnel Management						
b. Department, Agency or Establishment	Nurse	GS	0610	10	PM	02/28/2021
c. Recommended by Supervisor or Initiating Office	Nurse	GS	0610	10	CT	02/28/2021

17. Organizational Title of Position (*if different from official title*) Clinical Care Coordinator (PCMC)	18. Name of Employee (*if vacant, specify*)
19. Department of Health and Human Services	c. Third subdivision PHS Indian Medical Center, PHO/Phoenix Indian Medical Center (PIMC)
a. First subdivision Indian Health Service, Phoenix Area Indian Health Service	d. Fourth subdivision Division of Nursing Services, Ambulatory Nursing Services
b. Second subdivision Phoenix Service Unit	e. Fifth subdivision PCMC Nursing

20. **Supervisory Certification.** *I certify that this is an accurate statement of the major duties and responsibilities of this position and its organizational relationships, and that the position is necessary to carry our Government functions for which I am responsible. This certification is made with the knowledge that this information is to be used for statutory purposes relating to appointment and payment of public funds, and that false or misleading statements may constitute violations of such statutes or their implementing regulations.*

a. Typed Name/Title/Employee ID of Immediate Supervisor	*b. Typed Name/Title of Higher Level Supervisor or Manager (optional)*
Carolyn Tapahe, RN Supervisory Nurse, PCMC Nursing (Acting)	Candace Lee, RN, BSN, BC-CCTM Assistant CNO, Ambulatory Nursing Services
Signature Carolyn R. Tapahe -S Digitally signed by Carolyn R. Tapahe -S Date: 2021.02.10 13:33:58 -07'00'	Signature Candace Lee -S Digitally signed by Candace Lee -S Date: 2021.02.10 14:59:42 -07'00'

21. **Classification/Job Grading Certification.** *I certify that this position has been classified/graded as required by the Title 5, U.S. Code, in conformance with standards published by the U.S. Office of Personnel Management or, if no published standards apply directly, consistently with the most applicable published standards.*	22. Position Classification Standards Used in Classifying/Grading Position U.S. OPM Job Family Position Classification Standard for Professional Work in the Medical and Healthcare Group, 0600, Nursing 0610 series, September 2017; and General Schedule Leader Grade Evaluation Guide HRCD-5 June 1998 (cross-reference only).
Typed Name and Title of Official Taking Action Patricia Marquez, Classification Specialist SWR Human Resources, PIMC	
Signature Patricia A. Marquez -S Digitally signed by Patricia A. Marquez -S Date: 2021.02.17 17:29:01 -07'00'	**Information for Employees:** *The standards and information on their application are available in the human resources office. The classification of the position may be reviewed and corrected by the agency or the U.S. Office of Personnel Management. Information on classification/job grading appeals, and complaints on exemption from FLSA, is available from the human resources office of the U.S. Office of Personnel Management.*

23. Remarks: **Position is covered by P.L. 101-630 or P.L. 101-647 or both.** Incumbent must be suitable for Federal employment, as determined by fingerprint check results and appropriate background investigation. An incumbent found not suitable after hire will be removed from this position. UFMS: ☐ Yes ☑ No

Conditions of employment – this position requires, at a minimum, Level 5 – MBI moderate risk level background investigation since the contents of the PD meets the Public Trust position criteria.

CSC: 000-000-000

24. *Description of Major Duties and Responsibilities and Evaluation Statement (See Attached)*

POSITION TITLE: Nurse
ORGANIZATIONAL TITLE: Clinical Care Coordinator (PCMC)
PP/SERIES/GRADE: GS-0610-10
ORG LOCATION: DHHS, IHS, Phoenix Area, Phoenix Service Unit, Phoenix Indian Medical Center, Division of Nursing Services, Ambulatory Nursing Services, PCMC Nursing
FLSA STATUS: Non-exempt (Per MOA)
JOB CODE: PX3439

INTRODUCTION

This position is located at the Phoenix Service Unit, Phoenix Indian Medical Center (PIMC), Phoenix, Arizona, and can be assigned to any location throughout the medical center/Phoenix Service Unit, i.e. Nursing Services, Ambulatory Services, Outpatient Departments, and patient care teams associated with the Medical Home. The incumbent is a member of an ambulatory medical home care team with both medical provider staff and other health care staff (clinical and non-clinical). Incumbent ensures patients at the Phoenix Indian Medical Center receive quality patient centered care that is consistent with the patient-centered medical home principles, and that care is timely, appropriate, cost effective, sensitive and coordinated to affect desired clinical outcomes.

The incumbent primarily works weekdays, Monday through Friday; however, may be required to work weekends and some federal Holidays and is subject to call back.

MAJOR DUTIES AND RESPONSIBILITIES

Clinical Care Duties (75%)

Implements and initiates standardized disease management and health prevention guidelines across the patient care continuum. Assess patient's needs individually and/or as a group with the care team. Collects subjective and objective health status data from multiple sources, which includes the patient, family members, team members and documented records. Analyzes health status data in terms of the unique individual and/or disease related situation. Responsible for setting up and monitoring patients enrolled in disease specific home tele-health monitoring and communicating clinical information and changes to clinical plan between patient and provider. Prioritizes the information collected based on the patient's condition and preferences, the situation, and/or anticipated needs.

Develops a plan of care with the care team according to clinical practice guidelines with the patient, family, caregiver(s) and significant other(s). Identifies resources to integrate additional clinical practice guidelines to support continuity of care. Develops and maintains patient registries based on disease, health condition and/or health prevention initiatives. Integrates patient and family education into the delivery of care. Educational activities will recognize the age, gender, race, ethnicity, diversity, and cultural norms of the individual patient and incorporate the patient and family ability to understand and participate in the plan of care.

Evaluates the outcomes of patient care and modify the plan of care based on outcomes, individually or with the team. Evaluation includes documentation of the reassessment of patient status, evidence of patient and family participation and responses, and the rationale for the revisions in the plan of care. Participates in the designing improved clinical care delivery systems and identifies opportunities and set priorities to continuously improve nursing and organizational performance through data collection and monitoring activities set by nursing, the care team and the Service Unit. Initiates therapeutic interventions and administers medications as prescribed by medical providers.

Documents nurse-patient encounters in the medical record, electronically and/or paper and use available organizational technology to enhance care, efficiency, and safety, including automated medical records and other appropriate tool. Participates in staff meetings and performance improvement studies/activities. Attends relevant in-service and education programs. Applies current nursing concepts and findings from research and/or studies to practice. Recognizes urgent/emergent patient and clinic situations and initiate emergency medical procedures and complies with external disaster, internal disaster, bomb threat and fire plans and drills. Assists with obtaining lab results, medical tests, discharge summaries and other information within the hospital and other facilities and assists with arranging appointments, procedures and discharge planning. Uses appropriate medical supplies and/or equipment for the assigned clinic, such as 12-lead EKGs, fetal monitor, pulse oximeter, etc. and collects patient specimens, including blood draw, if needed. Processes lab specimens for transport or perform tests, such as waived tests, to obtain results.

Perform other related duties as assigned.

Coordination Lead Duties (25%)

Determines which member of the nursing/health care team will provide patient care and/or services to the patient. Interventions delegated to nursing/health care team members are based on their demonstrated competency, state practice acts, available evidence-based guidelines, regulatory and governmental standards and organizational policy.

Coordinates clinic flow within the clinical care team. Coordinates and evaluates the competency and outcomes of care delegated to technical and assistive personnel.

Instructs employees in specific task and job techniques and checks on work progress and reviews completed work for accuracy and completion.

Resolves simple informal complaints and refers serious and repetitive complaints to Supervisor. Reports performance and conduct issues and progress and training needs of employees to the Supervisor.

FES FACTORS

Factor 1 – Knowledge Required by the Position **Level 1-6**

Knowledge of health care regulations and standards such as the Joint Commission (TJC) and Centers for Medicare and Medicaid Services (CMS) Conditions of Participation.

Knowledge of a full range of professional nursing concepts, principles, and practices and experience with clinical practice and procedures to analyze the full scope of problems associated with providing appropriate, patient centered, cost effective patient care. This clinical knowledge is applied to solve chronic health care problems related to general medical and medical specializations.

Knowledge of disease management concepts including the use of clinical practice guidelines for performance outcomes.

Knowledge of evidence based primary, secondary, and tertiary prevention protocols as it pertains to disease management.

Knowledge of a broad base of current practice in various health care delivery systems, including outpatient, emergency care, inpatient and specialty care processes and current treatment regimens and modalities.

Knowledge of institutional, community and regional health care systems to serve as an authority in matters pertaining to disease management.

Working knowledge of the operation of health care financing and Purchased and Referred Care Services related to patient referral processes, including Coordination of Benefits, Indian Health Service and the Department of Health and Human Services (DHHS) regulations, as well as Medicare/Medicaid program benefits and health care services contracts and agreements.

Ability to be self-directed and establish liaisons with inpatient/outpatient case manager/discharge planners as well as other clinical care coordinators to coordinate services throughout the continuum of care.

Ability to practice and promote customer service by promoting a customer driven environment by working effectively with patients, families, medical and internal and external health care personnel.

Ability to provide individual and family counseling, guidance and health instruction to help patients understand disease prevention and health maintenance.

Incumbent must possess and maintain a current, valid, and active state license/registration as a professional nurse.

Factor 2 – Supervisory Controls **Level 2-4**

The SCN sets overall health care objectives and defines assignments. Independently able to provide nursing care/services, education and counseling, and responds to emergencies in accordance with policy and accepted professional nursing practices without specific instruction. Work is reviewed for effectiveness of meeting clinical and administrative outcomes and

compliance with nursing and medical requirements, professional standards and Service Unit objectives and initiates.

Factor 3 – Guidelines **Level 3-3**

Guidelines consist of established objectives, policies and procedures and accepted utilization management, nursing and medical practice literature. Incumbent is expected to make recommendations regarding the efficiency and efficacy of the guidelines; however, guidelines are not applicable in every situation and independent professional judgment is required to analyze, prioritize, select, implement and determine the quality and appropriateness of the care planned for the patient(s).

Factor 4 – Complexity **Level 4-4**

Independently performs difficult and complex reviews and analysis of clinical care team data to assess and evaluate effectiveness in implementing and compliance with clinical practice guidelines. Uses considerable resourcefulness and judgment to implement modifications and select appropriate clinical practice guideline(s). Develops new and innovative processes to improve clinical care team involvement in disease management, preventative screening, and utilization of guidelines.

Factor 5 – Scope and Effect **Level 5-3**

The purpose of the work is to maximize efficiencies of health care resources planning for the needs of patients in the most appropriate setting, and to provide a seamless continuum of care. The objective is to improve the quality of care, decrease fragmentation of services and enhance patient quality of life and cost containment.

Factor 6 – Personal Contacts **Level 6-2**

The incumbent's duties and responsibilities cross all organizational lines throughout the hospital, (including medical, nursing, administrative, etc.) and also include frequent contacts with care providers in other clinical areas, other Service Units, and with contract providers and other community resources.

Primary contacts are with patients and their families and a wide range of interdisciplinary healthcare professional and administrative staff, including physicians, other medical staff providers, nurses, discharge planners, case managers, social workers, pharmacists, physical therapists, nutritionists, business office staff, PRC service staff and others. These contacts are made within Ambulatory Nursing; the Phoenix Indian Medical Center, at other Service Units and with other public and private sector providers.

Factor 7 – Purpose of Contacts **Level 7-b**

Contacts are for the purpose of motivating patients undergoing disease management services; and requires tactful approach by incumbent to carry out duties including the development and implementation of clinical practice guidelines.

Factor 8 – Physical Demands **Level 8-2**

Work requires infrequently long periods of walking, standing, and recurring of heavy lifting, such as assisting patients and carrying items like boxes of manuals and files. Work may also require driving to field clinics or other healthcare agencies.

Factor 9 – Work Environment **Level 9-2**

Work is performed in Ambulatory Nursing Services with common exposure to contagious diseases. Some degree of hazard is present in working with disturbed patients. Field clinics and health care agencies differ in physical layout.

OTHER SIGNIFICANT FACTOR

Conditions of employment – this position requires, at a minimum, Level 5 – MBI moderate risk level background investigation since the contents of the PD meets the Public Trust position criteria.

FES EVALUATION
Nurse, GS-0610-10
Organizational Title: Clinical Care Coordinator - PCMC

Location: DHHS, IHS, Phoenix Area, Phoenix Service Unit, Phoenix Indian Medical Center, Division of Nursing Services, Ambulatory Nursing Services, PCMC Nursing

Background: This position has been realigned under the Division of Nursing Services from the Division of Medical Services, effective Feb/March 2021. The PD has been updated to reflect this reorganization. – *PMarquez*

Previously, on September 28, 2017 OPM released a new series for the 0600 professional occupations to include nursing positions. This position was previously evaluated using the superseded GS-0610 Nurse standard dated June 1977; and is being reviewed and re-classified using the new series as mandated by IHS HQs. The previous evaluation has been carried over and a follow-up evaluation is being attached to indicate what has changed and how the new series has impacted this position. Existing PD# PX3439 is shared by PCMC and Pediatrics; both remain the same and the grade level is not impacted. The previous established title is being changed from Nurse Specialist to 'Nurse' which aligns with the new series' titling practice.

On 7/9/2019 minor edits were made to existing PD but does not impact the grade level – PMz

Historical: An approved request to recruit packet was received on July 1, 2016 to backfill vacated position. The existing PD on file #PX3439 is occupied by other Clinical Care Coordinators and is categorized as 'non-exempt'. Due to the May 27, 2015 FLSA settlement between Indian Health Service and LIUNA, the memorandum of agreement has been abolished, effective March 7, 2016. However, per instructions and guidance from HHS Headquarters issued on November 29, 2016 all GS-610-09/10 positions are to be deemed "Non-Exempt". The Memorandum of Agreement has been reinstated effective November 29, 2016. The memo states ***"Effective immediately, you may consider the attached "Second Memorandum of Agreement LIUNA, NFFE, AFGE and IHS, dated May 21, 2010, to be in effect. All new GS-0610-09 and -10 Nurse Positions should be classified in FLSA Non-exempt status in accordance with this MOA. GS-0610-09 and -10 Nurse Positions recently classified as FLSA Exempt should be converted to Non-exempt status as soon as possible".*** The previous evaluation of this position has been carried over to ensure the classification meets the OPM standards

PD#: PX3439

References: U.S. OPM Job Family Position Classification Standard for Professional Work in the Medical and Healthcare Group, 0600 (covers 0610 Nursing), September 2017.

Title and Series Determination: The new 0610 series covers positions that manage, supervise, lead, and/or perform professional work in nursing. The work involves providing care to patients, promoting better health practices, and teaching or performing research, or consulting and advising nurses who provide direct care to patients. This position serves as a Clinical Care Coordinator in both PCMC and Pediatrics, and can be located throughout the medical center to

support the Medical Home concept. The nursing duties has not changed and therefore, the new series definition is a good fit for this position.

The General Schedule Leaders Grade Evaluation Guide (GSLGEG) was used to cross-reference the lead duties indicated in the position description at 25%; however, because the lead duties does not meet the minimum authorities and responsibilities as outlined in the GSLGEG's Part II; as not meeting the first seven coaching, facilitating, and mentoring duties and a total of fourteen of the twenty duties listed; the prefix "Lead" was not required in the assigned title.

The basic title referenced in the new 0610 series, authorizes the use of "*Nurse*" for positions whose work involves protection, promotion, and optimization of health and abilities, prevention of illness and injury, facilitation of healing, alleviation of suffering through the diagnosis and treatment of human response, and advocacy in the care of individuals, families, groups, communities, and populations. Work typically involves providing direct nursing services to patients in a variety of settings such as hospitals, clinics, outpatient clinics, and emergency rooms. The "*Advanced Practice Nurse*" title is used for positions serving as a primary or specialty care provider, independently or on consultative basis with physicians, to provide direct comprehensive nursing care, therapeutic procedures, and preventive health education and maintenance. The APN title is used for Nurse Practitioners, Certified Nurse Midwives, Clinical Nurse Specialist and Certified Nurse Anesthetist. Of the two titles covered, this position performs work that is not advanced in nature and therefore, the official title that best fits is **"*Nurse*"**. The organizational title of *Clinical Care Coordinator* can be used for internal agency use or for recruitment purposes.

Grade Level Determination: The new 0600 series provides evaluation using the Factor Evaluation System as previously evaluated to include: (1) Knowledge Required by the Position; (2) Supervisory Controls; (3) Guidelines; (4) Complexity; (5) Scope and Effect; (6) Personal Contacts; (7) Purpose of Contacts; (8) Physical Demands; and (9) Work Environment.

1. Knowledge Required by the Position - *measures the nature and extent of information or facts which the nurse must understand to do acceptable and the nature and extent of the skills needed to apply those knowledge.*
Remarks: The knowledge required by the subject position exceeds FL 1-5 since this level only requires basic knowledge of professional nursing concepts, principles, and practices in order to support the basic preparation in professional nursing. According to the PD, the position fully meets FL 1-6 as it is evident a full range of professional skill and knowledge of established concepts, principles, and practices is needed to perform professional nursing assignments of moderate difficulty nursing care; and in advising health care needs, with full consideration of mental, emotional, cultural, social and physical factors truly exists. In addition, knowledge of institutional, community and regional health care systems pertaining to disease management is needed in order to provide continuum of care for the individual and family. FL 1-7 is not met because at this level knowledge required by the position is used to perform highly specialized degree of skills and ability in a particular field; whereas in this case, knowledge of a broad base of current practice, including outpatient, emergency care, inpatient and specialty care processes applies. Therefore, **FL 1-6 is credited for 950 points.**

2. Supervisory Controls - *covers the nature and extent of direct or indirect controls exercised*

by the supervisor, the employee's responsibility, and the review of completed work.
Remarks: The supervisory controls for the subject position clearly exceeded FL 2-3 and is most comparable to FL 2-4, such that the position reports to the Clinic Practice Manager but receives clinical oversight of nursing practice from the Nurse Mentor/Lead about new procedures, improved nursing techniques and patient flow. The position independently performs difficult and complex reviews and analysis of clinical care team data and provides nursing care/services, education and counseling, and responds to emergencies without specific instruction. Finally, work is reviewed for effectiveness of meeting clinical and administrative outcomes and compliance with nursing and medical requirements. FL 2-5 is not met since the position is not responsible for planning, designing, and carrying out programs or projects and does not make recommendations for new projects and alteration of objectives. Therefore, **FL 2-4 is credited for 450 points.**

3. Guidelines - *covers the nature of guidelines and the judgment needed to apply them.*
Remarks: The guideline for the subject position fully meets FL 3-3 since guidelines are established and are made available; and guidelines are not applicable in every situation and independent professional judgment is required to analyze, prioritize, select, implement and determine the quality and appropriateness of the care planned for the patient(s). FL 3-4 is not met since there are no limitations of guidelines that will eventually place the position in a situation where the position will have to propose new policies as indicated in the guide. Therefore, **FL 3-3 is credited for 275 points.**

4. Complexity - *covers the nature, number, variety, and intricacy of tasks, steps, processes, or methods in the work performed; the difficulty in identifying what needs to be done; and the difficulty and originality involved in performing the work.*
Remarks: The complexity for the subject position exceeds FL 4-3 and FL 4-4 is fully met since the position independently performs difficult and complex reviews and analysis of clinical care team data to assess and evaluate effectiveness of clinical practice. The work also requires making decisions concerning the implementation of data, planning and refining methods by developing new and innovative processes to improve clinical care team involvement in disease management. FL 4-5 is not met since the work does not require many different and unrelated processes or substantial depth of analysis; nor does the work include major areas of uncertainty in approach. Therefore, **FL 4-4 is credited for 225 points**.

5. Scope and Effect - *covers the relationship between the nature of the work, i.e., the purpose, breadth, and depth of the assignment, and the effect of work products or services both within and outside the organization.*
Remarks: Scope and effect for subject position exceeded FL 5-2 and is most comparable to FL 5-3, because the purpose of the nurse's work is to plan and provide nursing care for patients that maximize efficiencies of health care resources in the most appropriate setting, and to provide a seamless continuum of care. The objective is to improve and enhance patient quality of life and cost containment for patients and their families. This factor level does not meet 5-4 because position is not participating in the establishment of new program or the evaluation of program effectiveness. In conclusion, **FL 5-3 is credited for 150 points.**

6. Personal Contacts - i*ncludes face-to-face contacts and telephone and radio dialogue with persons not in the supervisory chain.*

Remarks: Personal contacts for the subject position exceeds FL 6-1 since personal contacts extend beyond the immediate organization and care providers from other Service Units, contract providers to include public and private sectors, and other community resources outside the organization; therefore, FL 6-2 is fully met and does not meet FL 6-3 because contacts do not include manufacturers, professors, attorneys, scientists, representatives of professional or trade organizations, the news media, and organized or ad hoc public action groups. Therefore, **FL 6-2 is credited.**

7. Purpose of Contacts - *ranges from factual exchanges of information to situations involving significant or controversial issues and differing viewpoints, goals, or objectives.*
The purpose of contacts is most comparable to FL 7-b. The purpose of contacts is to plan, coordinate, or provide case management/care coordination services. Also to motivate patients undergoing services that may be skeptical or hesitant. **FL 7-b is credited.**

On Page 72 of the new 0600 series, FL 6-2 and FL 7-b intersect and is credited 75 points.

Point Assignment Chart					
		Purpose of Contacts			
	Level	A	B	C	D
Personal Contacts	1	30	60	130*	230*
	2	45	75	145	245
	3	80	110	180	280
	4	130*	160	230	330

8. Physical Demands - *covers the requirements and physical demands placed on the employee by the work assignment.*
Remarks: Physical demands for subject position exceeds FL 8-1 and fully meets FL 8-2 since the work requires long periods of walking, standing, and recurring of heavy lifting, such as assisting patients and carrying items like boxes of manuals and files. FL 8-3 is not met because work does not involve strenuous physical exertion or heavy lifting over 50 pounds. Therefore, **FL 8-2 is credited for 20 points.**

9. Work Environment - *considers the risks and discomforts that may be imposed upon employees by various physical surroundings or job situations.*
Remarks: Work environment for subject position is most comparable to FL 9-2 since the work is performed in Ambulatory Care where position's work environment poses risk of exposure to contagious diseases. Some degree of hazard is present in working with disturbed patients. Field clinics and health care agencies differ in physical layout. FL9-3 is not met since work environment does not involve potentially dangerous situations or unusual environmental stress where conditions cannot be controlled. Therefore, **FL 9-2 is credited for 20 points.**

EVALUATION REVISITED - SUMMARY

FES Factors	Level Credited	Points Assigned
1. Knowledge Required by the Position	1-6	950
2. Supervisory Controls	2-4	450
3. Guidelines	3-3	275

4. Complexity	4-4	225
5. Scope and Effect	5-3	150
6. Personal Contacts	6-2	75
7. Purpose of Contacts	7-b	
8. Physical Demands	8-2	20
9. Work Environment	9-2	20
SUMMARY	**Points Total:**	**2165**
	Grade Conversion:	**GS-10 (2105-2350)**

Approval Organizational Title: Clinical Care Coordinator (PCMC)

Final Allocation (New Title): Nurse, GS-0610-10

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Classification Specialist Date
Southwest Region Human Resources-PIMC